From The Operator's Thesis: How Healthcare Really Buys AI (Saivai Group, 2026) — editable reader copy of the printed instrument; text identical to the book. © 2026 Anthony W. Russell. For the reader's own use; not legal, clinical, compliance, investment, accounting, tax, or consulting advice. Reader resource hub: saivaigroup.com/thesis

Appendix A — The Operator's Toolkit

The instruments below convert this book's frameworks into working diligence and governance tools. All seventeen appear here in full. Three of them, the sheets most readers will use first, appear as fill-in worksheets and a checklist; the rest are deliberately compact prose, because an instrument you can hold in one read is an instrument you will actually use. These instruments are aids to structured thinking based on the author's operating experience. They are not legal, clinical, compliance, investment, accounting, tax, or consulting advice; they do not establish a standard of care or guarantee diligence or outcomes. Use them with qualified professionals and your own judgment. See the Author's Note.

Before the Instruments: The Small-System Start

If the process in this book reads like it was built for a system with forty people to throw at it, it was, because that is who the enterprise gauntlet was built by. If you are smaller, do not run the gauntlet. Run these five moves; each points to the one instrument that carries it.

1. One owner, one page. Name the sponsor, signer, implementer, and user: four names, one page (Instrument 5). If two of those names are the same person, write it down anyway; the vacancies are the finding.
2. Price the whole bill. One hour with whoever holds your budget and the Price-Plus-Burden worksheet (Instrument 3). If the vendor cannot help you fill in the right-hand column, that is your answer.
3. Charter before pilot. Six lines in a table (Instrument 8). If any line cannot be filled in, the pilot is not ready, and that discovery is free today and expensive in month four.
4. Round on the change, weekly. The checklist's standing six (Instrument 16) takes fifteen minutes and replaces the meeting nobody scheduled.
5. Book the renewal decision before go-live. Put the date, the owner, and the evidence you will require on the calendar now (Instrument 11). The renewal is the verdict; schedule the courtroom.

That is the whole starter path. An enterprise process does not scale down; judgment does, and these five moves are the judgment.

Instrument 1 — The Buyer Stack Worksheet *(Chapters 4–5)*

For any healthcare AI purchase under evaluation: by a vendor mapping a deal, an executive governing one, or an investor diligencing one — name a human for every row (the seven seats of Figure 3). Unfilled rows are the risk report.

Seat	Who, by name	What they need before yes	What they risk by yes
Champion			
Sponsor			
Signer			
Budget owner			
Implementer			
User(s)			
Renewal owner			

Scoring is unnecessary; the empty cells are the finding. Three diagnostic questions follow: Which seat is doing double duty, and can they? Which function will resist hardest, and is it about the product or the burden? Who will be in the renewal meeting, and what will they remember?

Instrument 2 — The Three Fits Scorecard *(Chapter 10)*

Score each fit 1–5 for a named product in a named segment. A product is durable only where all three clear 4.

Workflow fit: Net effort at the point of use is negative within days. The product conforms to the workflow's rhythm or transforms it with a credible change plan. Expert, motivated users stay. *(My half-day test: would this have kept me seeing patients?)*

Economic fit: Value lands in a nameable budget, on a stated timeline, surviving the CFO's three questions: replacing or adding spend; where the return shows up, whose budget, how fast; realistic odds implementation erases it. Price-plus-burden accounting attached.

Trust fit: Safety architecture demonstrable at Donabedian's three levels (structure, process, outcome). Autonomy graduated, supervised, revocable. Regulatory posture settled pre-pilot. Culture fit assessed: this institution's actual change capacity, not the segment's average.

Failure-mode key: high workflow + economic, low trust = *ungovernable*. High workflow + trust, low economic = *unfundable*. High economic + trust, low workflow = *unused*.

Instrument 3 — The Price-Plus-Burden Worksheet *(Chapter 6)*

Left column of your spreadsheet: the invoice. This page is the right column: the bill the invoice never shows, priced in hours and named roles. Fill it in before the budget meeting, not after.

The burden line (beyond the invoice)	**Who carries it (name the role)**	**Year-one cost (hours × rate)**
Implementation		
Integration and interfaces		
Training — initial and ongoing		

The burden line (beyond the invoice)	Who carries it (name the role)	Year-one cost (hours × rate)
Security, legal, and governance review		
Exception handling at steady state		
Support and escalation		
The champion's political capital		
Leadership attention		
Total: price + burden		

Then three questions: What does the vendor say this costs the institution in year one, and is the answer a blank look? Who carries each line, by role? What does "free" actually consume? (Pilots score here too; Chapter 6's rule: nobody prices the institution's attention, so this sheet does.)

Burden-shift check: every automation must name who gets relief, who inherits work, and whether the enterprise delta is positive after verification, exceptions, reconciliation, training, monitoring, and renewal defense.

Instrument 4 — Diligence Questions by Stage *(Chapters 11–12)*

Pair every vendor-clock milestone with its buyer-clock gate. Early story: has any health system moved this past curiosity into genuine scrutiny? First major round: a real security review and governance process cleared on the merits — where, and may I call them? Next round: implementation survived beyond design partners; where did the burden land, mapped by whom? Growth capital: renewals — how many, at what price, with the champion still required in the room or not? Expansion claims: arriving in the customer's voice (invitations) or the sales team's (pushes)? Always: what does the deployment cost the *company* per month in people, and what happens if the founder doesn't visit for a quarter?

Instrument 5 — Sponsor / Signer / Implementer / User Map *(Chapter 4)*

The rapid version of Instrument 1, for a single live deal. Write four names. The sponsor: who lends this executive legitimacy, and what do they risk by doing so? The signer: who authorizes the contract, and what evidence do they need that the sponsor cannot supply? The implementer: who inherits the deployment, and have they attended a single meeting yet? The user: whose daily behavior decides whether value exists, what does the product cost them at the point of use, and can they sustain the new workflow without drifting back to the old one? Then the test: how many of the four believe they are the buyer, and does the vendor know which one actually is?

Instrument 6 — Safety Language vs. Safety Architecture Checklist *(Chapter 8)*

Take every safety word in the vendor's materials and walk it back to its mechanism, demanding evidence at Donabedian's three levels. *Accountable*: what exactly is the human accountable for, and does the workflow give them a realistic way to exercise it — show the review surface.

Transparent: can someone reconstruct, after the fact, what the system did and why — show the trace. *Monitored*: what is measured, against what thresholds, and who receives the alert overnight — show the dashboard and the on-call rota. *Governed*: what can be constrained, audited, and defended in front of a board — show the controls. *Compliant*: BAAs, minimum-necessary, breach obligations, consent and disclosure workflows, FDA status if applicable — show the artifacts, dated before the pilot. *Validated*: validated where, on whom, and who is watching it now — show the local numbers. A vendor fluent at the document level only has compliance theater. Structure, process, outcome, or it is language.

Instrument 7 — Judgment Allocation Map *(Chapter 7)*

For each task in the target workflow, assign a category and a supervision level. Category: machine-favored (high-volume, pattern-heavy, errors detectable and recoverable — run with audit); human-led, machine-assisted (machine drafts and narrows, human decides: the resident-and-attending structure); human-dominant (ambiguity, ethics, or accountability make delegation impossible: AI supports the surroundings only). Supervision level, per the Attending Model: observe; propose with co-signature; act with review; act with retrospective audit; act independently within a bounded domain: each granted by domain, earned by demonstrated performance, and revocable. Then name the two humans in the loop: the frontline lead who manages the process where it runs, and the accountable administrator who owns value, burden shift, compliance, and drift at the executive level. Finally, the training plan: no human enters the loop untrained — what does each role learn, when, and how is competence checked? If the vendor's answer to the human's role is "rescue" rather than "judgment," you are pricing a services business, not a safety architecture.

Instrument 8 — Pilot-to-Platform Readiness Checklist *(Chapter 12)*

The pilot charter — the five lines of Chapter 12, set as a six-row template. Before anything else, confirm the pilot has a charter. If any row cannot be filled in, the pilot is not ready to start.

Charter line	**Enter it here**
Named owner — institution side	
Named owner — vendor side	
Scope and end date	
Success criteria — including burden criteria — agreed in writing before go-live	
Measurement plan finance has blessed in advance	
Decision path: what converts the pilot, what kills it, and who makes that call	

Wedge criteria: the product runs in real production, past the demo cohort and the design partners; the institution would operationally notice if it were turned off; the burden has been mapped and landed where the vendor said it would; the first renewal has been earned at full price.

Trust-transfer signals: references volunteer themselves; an adjacent department has asked for the product; the champion is no longer needed in the room for the renewal conversation.

Implementation-theater screens, asked of every flagship deployment: What does this deployment cost the vendor per month, in people? What breaks, operationally, if the founding team stays home for a quarter? How many deployments run untouched by them? Who at the customer would fight to keep this if the champion left tomorrow? A wedge survives all four. Theater survives none.

Instrument 9 — Founder Red Flags *(Chapters 11–12, 15)*

Platform language before wedge durability. Pilot counts offered where renewal counts were asked for. Burden questions answered with pricing slides. Compliance treated as the customer's problem. Resentment of the courtship — references, advisors, accelerator vetting — that first health-system relationships legitimately require. Heroics presented as repeatable product performance. Expansion that happens only where the sales team pushes. A market map where the diligence question was about one buyer's coalition. And the meta-flag that contains the others: the inability to answer, simply, *what are you trying to accomplish, and how does this actually get you there?*

Instrument 10 — Buyer Red Flags *(Chapters 3, 4, 9)*

The telltale flags, in the chapter's own terms: no AI governance lane; no named executive owner; access without training; tools distributed through the help desk.

For vendors qualifying accounts and investors qualifying their companies' revenue. No AI governance lane — no intake, no criteria, no supervision levels — so decisions will be made by altitude and mood. No named executive owner for the deployment. Access without training plans; tools distributed through the help desk; nobody tracking who has what. A CEO-sourced vendor proceeding without comparison. Scar tissue unexamined: nobody has asked what the last three vendors in this category left behind. A pilot accepted with no success definition, no end date, and no owner. Revenue signed into these conditions has no anchor under it: price it accordingly.

Instrument 11 — Renewal Readiness Questions *(Chapters 6, 11)*

To be answered six months before the meeting, by the vendor and the institution both. Where did the promised value actually land: in whose budget, measured by whom, against what baseline? What does the burden audit show: who got relief, who inherited work, and what does the net look like at the enterprise level? What broke, and how did support actually perform when it did? Who at the customer would fight to keep this — by name — and is the original champion still required for the case to hold? What will institutional memory say in the room? If the vendor cannot answer these, the renewal is a coin flip. If the institution cannot, its governance has stopped governing.

Instrument 12 — The Healthcare AI Company Quality Scorecard *(Chapter 20)*

Score the five marks, 1–5, with evidence required for anything above 3. Real value where the money decides: economic legibility, burden-honest accounting, value surviving a renewal. Institutional trustworthiness: architecture over language; graduated, supervised, revocable autonomy; compliance fluency predating the first pilot. Operating fit: the three fits assembled in a named segment, with honest boundaries. Earned expansion: wedge in production, renewals at full price, growth arriving by invitation. Mission-level significance: the institution is better at being a healthcare institution — for the patients receiving care and the people delivering it, because this company exists. Companies scoring high on the first four and low on the fifth are extracting. The reverse are dreaming. Back the ones that refuse the choice.

Instrument 13 — The AI Team Criteria Checklist *(Chapter 13)*

Before any purchase, name a human for every seat; an unstaffed seat is the risk report. The seven seats, in the chapter's terms: Executive owner; Translator-operator / clinical workflow interpreter; Clinical workflow lead; IT/security partner; Analyst/build talent; Training and change lead; Finance/value partner. Executive owner (CEO-mandated, holds budget and barriers). Translator-operator / clinical workflow interpreter — test the candidate against the profile, not the résumé: can they sit with a physician and hear what is underneath the complaint; move fluently between clinic flow, front-desk reality, IT constraint, and finance scrutiny; turn executive direction into a running pilot; do they hold the users' trust? Clinical workflow lead with genuine frontline credibility. IT/security partner who can make it survivable. Analyst/build talent fluent in both the system and the clinic it serves. Training and change lead who owns adoption itself. Finance/value partner who will count what the renewal will ask about. Then the two humans in the loop, staffed by name before go-live: the frontline lead inside the work; the accountable administrator above it. Failure flags: ownership defaulting to the CIO; the champion structurally alone; capability acquired with no owner for competence; access distributed through the help desk; a roster that exists only on the kickoff slide.

Instrument 14 — The Process-Fit Worksheet *(Chapter 14)*

Answer before the pilot, with numbers. The population: What percentage of the target panel is Medicaid? What languages are required, at what volumes? What is portal adoption by segment, not the enterprise average? Which communication channels actually connect (phone, SMS, portal, letter, family navigator), by segment? What are the no-show and referral-leakage patterns? Who currently closes the loop manually, and what happens when a patient does not respond digitally? Does the workflow escalate gracefully to a human, or strand the patient? The operating question for every patient-facing workflow: does this tool help the patient get to the next right step, or does it move staff burden onto the patient? The model's population: What population was this trained and validated on? Show performance broken out by payer mix, language, and age. Will it learn our population first, or impose the process its training assumed? What happens to the published metrics when portal adoption is half the flagship's? The institution: Which units have the change capacity to absorb this now? What does the system landscape (EHR, workqueues, phone tree, scheduling) require of the integration? Scoring rule:

a tool may pass on the vendor's population and fail on yours; only yours counts. Closing question, asked out loud: does this workflow narrow the gap or widen it?

Instrument 15 — The Product Differentiation Scorecard *(Chapter 15)*

Run this for two or three comparable products: never one; a single-vendor evaluation can only answer "is this impressive?"

1. **Weighted criteria, before any demo.** Anchor them to your patients and your operation, and rank and weight them before vendor narratives arrive. Mark each as a must-pass gate or a differentiator. Criteria written after the demos are rationalizations with a rubric.

2. **Comparison set.** Two or three products minimum, never one. Only comparison answers "is this better, for us, at this price, with this burden?"

3. **Team composition.** Not just executives and reviewers but the people who do the work the technology will touch, including the translator-operator and frontline reviewers, who become the change's champions afterward.

4. **Reference-call question set** (real references, beyond the curated list): What did this actually feel like inside your health system? What worked with that team that was genuinely great? What didn't work? What were the challenges, and how did you overcome them, and how did they help? Were there problems they couldn't address, and who finally did? How did it turn out, and how did the whole experience feel in the end: any negative feelings, and about what?

5. **The master question:** *What has to happen inside our system for the value in this demo to become real, and who carries each step?* Walk it with named carriers and honest hours across vendor, IT, operations, physicians, nurses and MAs, compliance and legal, training, finance, and patient.

6. **Three follow-ups:** Where might the saved work reappear? What would make us regret this purchase in a year? If the founding team disappeared from this deployment tomorrow, would it still work?

7. **The final buyer question:** *Which product can our institution live with, support, govern, and renew?*, not which performs best in the demo.

8. **The investor reversal.** Simulate the customer's bake-off: ask which structured comparisons the company has won, which it has lost, and — most revealing — why it lost, in the company's own telling.

9. **The why-it-lost log.** For each bake-off lost, record the buyer segment, the winning alternative, the criterion that decided the loss, the operational reason behind it, and whether the loss reflects product weakness, buyer mismatch, pricing/burden, governance maturity, integration/supportability, or customer readiness.

Score independently against the weighted criteria, then deliberate as a group; the disagreements are the findings.

Instrument 16 — The Change Readiness Checklist (for Buyers) *(Chapter 16)*

	Check each item; an unchecked box is an action, not a footnote.
	Hardwiring
☐	Access granted only with training attached
☐	Rollout sequence written before the pilot
☐	Stop conditions defined and announced
	Leadership
☐	Executive owner named, with a weekly cadence
☐	Executives trained first
☐	Leaders can use the tool they are sponsoring
	Rounding for outcomes — weekly during pilot
☐	What is easier
☐	What is harder
☐	Where did work move
☐	Are you checking the output, and what are you finding
☐	What errors or risks appeared
☐	What training gaps surfaced
	The busy leader's standing six
☐	Who is using it
☐	Who is not
☐	What burden moved
☐	What error appeared
☐	What training gap surfaced
☐	What do we stop, start, or adjust
	Communication
☐	Key words at key times: a written script for what is changing, what is not changing, what staff remain accountable for, what support exists, and what triggers a halt — delivered by leaders, not by email
☐	Say it three times: the change communicated as possibility, then direction, then decision, with honest spacing
	Capacity
☐	Unit-by-unit change-capacity check before sequencing
☐	Recognition plan for early adopters

	Check each item; an unchecked box is an action, not a footnote.
☐	A defined moment when pilot becomes decision
☐	Leadership inspectability: can leaders demonstrate the behavior they are asking staff to adopt?
☐	Change-capacity sequencing: which units should not receive this tool yet because they are already carrying too much change?

Instrument 17 — The Pilot-System Selection Checklist (for Vendors) *(Chapter 16)*

Underwrite the institution the way an investor underwrites you. Ownership: who owns this internally, and have you met them, not heard about them? Is the champion's chain of command engaged and aligned *before* the pilot, starting with the executive the champion reports to? Path: what process will evaluate the pilot; who can say yes afterward; who can say no; is the pilot tied to a buying decision or to curiosity? Criteria: what converts this pilot; what kills it even if users love it; are success metrics agreed in writing up front? Capacity: does the institution have the training and change capacity to deploy honestly; is there a governance lane, or leadership willing to build one? Truth test: will this customer teach us repeatable truth or custom chaos — will their reference call (Instrument 15) someday be one we would want taken? Red flags: no executive owner; no decision path; champion structurally alone; senior leadership able to override the closest operator with no defined evaluation; "innovative" reputation with no named owner; a pilot accepted with no end date. Rule: the wrong pilot customer gives a startup false confidence; the right one gives it reusable truth. Champion path: does the champion have a path? First-customer training effect: what will this customer teach our company? Reference-call future: would we want this institution to be the reference call Chapter 15 asks buyers to make?